A/PROF CHAMARA BASNAYAKE

MBBS, PhD, FRACP

Gastroenterologist & Endoscopist

PN 421926PK

FOCUS GASTROENTEROLOGY

14 May 2026

Dr Magdy Sedrak
St.Luke Medical Centre

Dear Dr Sedrak,

Re: Miss Vanessa MALEKO
DOB: 13/05/1998 Phone: 0405081875
Address: 17 Mavora Court, TAYLORS HILL, VIC, 3037
Email: vanessasawannn@gmail.com

Summary

- **Improvement in abdominal pain and diarrhoea frequency on current medical therapy.**
- **Ongoing symptoms of heartburn and reflux post-sleeve gastrectomy.**
- **Escalation of nortriptyline dose to address suspected visceral hypersensitivity.**
- **Surgical conversion to gastric bypass deferred until after future pregnancy.**
- **No action required by you.**

I had the pleasure of conducting a review with Vanessa. She reports that her original symptoms of abdominal pain and diarrhoea are now significantly less frequent and less severe than her pre-treatment baseline. While she still experiences flare-ups every few days, the daily constant pain has subsided.

The primary issue at present is persistent heartburn. Vanessa feels that her current acid suppression—consisting of rabeprazole and nizatidine—is not providing adequate coverage, with symptoms often "wearing off" by the afternoon. We discussed the possibility that this represents visceral hypersensitivity to small amounts of reflux, rather than simple acid breakthroughs. I also noted that in a very small minority of patients (approximately 2%), nortriptyline can paradoxically contribute to a sensation of heartburn, though I suspect hypersensitivity is the more likely driver here.

Vanessa has recently consulted with Mr Matthew Read regarding a potential conversion to a gastric bypass to manage her symptoms and a suspected hernia. However, she is planning a pregnancy for mid-next year and wishes to avoid major revisional surgery until she has completed her family. She expressed concern regarding the management of reflux during pregnancy. I have reassured her that we have a variety of safe and effective acid suppression options to use during that period, and we would likely pause the nortriptyline during pregnancy as a precaution.

Impression and Plan My impression is that we should continue to lean on medical management while she pursues her family goals. I have encouraged Vanessa to increase her nortriptyline from 25 mg to 50 mg (two tablets) at night. She was initially hesitant due to concerns regarding sedation, but I have explained that nortriptyline is generally much better tolerated than amitriptyline in this regard. She will trial this higher dose for the next month to determine if it improves her oesophageal sensitivity. I have provided her with repeat prescriptions for rabeprazole and nortriptyline. I will review her again in approximately five weeks to assess

All appointments and enquires

Suite 201, 100 Victoria Parade
East Melbourne VIC 3002
Ph: 03 9650 7917
Fax: 03 9650 7910
Pager: 03 9387 1000
Email: office@focusgastro.com.au

Endoscopy locations

Epworth Freemasons Day Proc. Centre
320 Victoria Pde, East Melbourne, 3002
Hobson Healthcare Sydenham
566 Melton Hwy, Sydenham, 3037
St Vincent's Private Hospital
59 Victoria Pde, Fitzroy, 3065

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her response to the dose escalation.

Yours sincerely,

A/Prof Chamara Basnayake
Gastroenterologist

CC: Prof Matthew Read

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